

Policy on Transfer In, Transfer Out and Referral

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Emergency In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Patients who arrive by transfer, leave by transfer or referral, or move for diagnostics do so safely — stabilised where needed, accompanied by the right staff, with a written summary of condition and treatment.

1. Purpose

This policy says how «Hospital Name» receives transfer-in patients appropriately (planned and unplanned); transfers out or refers patients to another facility appropriately; assigns accompanying staff appropriate to the clinical condition; and gives a summary of the patient's condition and treatment given.

The chapter intent is that emergency patients receive life-stabilising treatment and are then either admitted or transferred appropriately, and that transfer and discharge protocols are well defined.

This policy does not cover in-hospital registration, admission, routine discharge-summary content, or continuity of care within the organisation — those are covered in other hospital policies.

2. Scope

This policy applies to emergency, wards, intensive care, day-care and any unit at «Hospital Name» that receives a transfer-in or initiates transfer-out or referral. It binds treating doctors,

accompanying nurses and technologists, ambulance coordinators, registration staff who receive transfer-in patients, Medical Records, the Medical Superintendent, and the Quality Coordinator.

It covers the organisation's policy and procedure for transferring patients in from other facilities, transferring patients out to other facilities, and referring patients — including pre-transfer contact with the receiving facility, clinical information accompanying the patient, and transfer documentation.

Boundaries:

- Bed non-availability holding is covered in other hospital policies; once the decision is to transfer out, this policy owns the transfer.
- Discharge-summary content for patients discharged and transferred (including LAMA) is covered in other hospital policies; this policy requires that a discharge or transfer summary is given and a copy retained.
- COP standards own clinical stabilisation techniques; this policy owns that stabilisation, mode, equipment, monitoring and accompanying staff are decided and documented before transfer.
- Lab and imaging own the receiving diagnostic service when a patient is shifted for tests; this policy owns safe transfer to that service when it is outside the immediate unit.

3. Policy standards

«Hospital Name» receives transfer-in patients appropriately for both planned and unplanned transfers. Transfer-out and referral to another facility are done appropriately, including pre-transfer stabilisation where appropriate, choice of mode and vehicle, equipment and monitoring, with gaps documented when requirements cannot be met. Accompanying staff match the patient's clinical condition. A transfer summary — or a discharge summary when the patient is discharged and transferred, including leaving against medical advice — is given, and a copy is retained.

«Hospital Name» does not transfer an unstable patient without meeting transfer criteria, completing appropriate stabilisation, and assigning appropriate accompanying staff.

4. Non-negotiable rules

1. Do not accept or dispatch a transfer-in without recording planned versus unplanned status and the clinical information received.
2. Do not transfer out or refer a patient without consulting the patient and/or family where they can be consulted, and without addressing stabilisation, mode, equipment and monitoring — or documenting why a stated requirement could not be met.
3. Do not send an unstable admitted patient for transfer-out or for diagnostic shift without a doctor accompanying.
4. Do not send any transfer or referral without accompanying staff who are at least a trained trauma technologist, emergency technologist or nurse with basic or advanced

cardiopulmonary resuscitation training as appropriate, and who know the transfer procedure.

5. Do not complete a transfer without giving a transfer summary (or discharge summary if discharged and transferred, including LAMA) and retaining a copy.
6. Staff who find transfer rules not followed report it the same shift to the treating doctor or the Medical Superintendent.

5. What we do

5.1 Transfer-in done appropriately

Transfer-in covers planned and unplanned arrivals. For unplanned transfers and suspected unstable patients, «Hospital Name» may send a suitably trained person with the ambulance, guided by the information received. Feedback on the patient's clinical status is provided to the referring organisation or doctor as good practice.

On arrival, registration under the hospital's registration policy applies. The receiving doctor documents the transfer-in, clinical status on arrival, and whether the transfer was planned or unplanned. The Emergency In-Charge owns the unplanned transfer-in pathway; department heads own planned specialty transfer-ins.

5.2 Transfer-out and referral done appropriately

Patients needing transfer-out or referral include those who present to emergency but need another organisation, those already admitted who now need care elsewhere, and patients being shifted for diagnostic tests outside the unit's immediate capability.

Transfer is done in consultation with the patient and/or family, in a safe manner that includes:

- pre-transfer stabilisation where appropriate;
- choice of mode and vehicle for transport;
- equipment required during transfer;
- monitoring required during transfer.

If «Hospital Name» cannot meet some of these stated requirements, the reasons are documented in the transfer record before the patient leaves. The treating doctor authorises the transfer-out or referral.

5.3 Accompanying staff appropriate to clinical condition

Staff accompanying a transfer or referral are at least a trained trauma technologist, emergency technologist or nurse. That person has undergone training in basic or advanced cardiopulmonary resuscitation as appropriate to the role, and knows the transfer procedure.

A doctor accompanies an unstable admitted patient who is being transferred out or being shifted for diagnostic purposes. Stability is judged by the treating doctor against airway, breathing, circulation, conscious level and any specialty-specific instability criteria written in the transfer guidance.

The names and roles of accompanying staff are recorded on the transfer summary.

5.4 Summary of condition and treatment given

«Hospital Name» gives a transfer summary stating significant findings and treatment given to every patient transferred from the emergency ward or transferred for diagnostic or therapeutic purposes.

When a patient is discharged from the organisation and transferred out, a discharge summary is given — including patients leaving against medical advice (LAMA). A copy of the transfer or discharge summary is retained by the hospital in the medical record.

Medical Records verifies that the retained copy is filed under the unique identification number before the record is closed for that episode.

6. Stop-work authority

Do not transfer an unstable patient out of «Hospital Name», or shift an unstable admitted patient for diagnostics, unless transfer criteria are met, appropriate pre-transfer stabilisation has been done, and accompanying staff appropriate to the clinical condition (including a doctor for an unstable admitted patient) are assigned and ready.

If those conditions are not met, stop the transfer, continue stabilisation and escalate to the treating doctor and the Medical Superintendent the same shift. Document why the transfer was held.

Stop-work does not block emergency life-saving measures. It blocks unsafe transfer movement until criteria, stabilisation and accompanying staff are in place.

Refusing an unsafe transfer is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for transfer-in/out and referral written guidance and stop-work enforcement. Approves ambulance and accompanying-staff arrangements.
Treating doctors	Authorise transfer-out and referral; judge stability; accompany unstable admitted patients; write or approve transfer/discharge summaries.
Emergency In-Charge / department heads	Own unplanned and planned transfer-in pathways respectively. Ensure feedback to referring organisations where practicable.
Accompanying nurses / technologists	Hold current CPR training appropriate to role; know the transfer procedure; monitor during transfer.
Registration / front office	Register transfer-in patients per the

	registration policy; assist family communication for transfer-out.
Medical Records	Retain copies of transfer and discharge summaries under the unique number.
Quality Coordinator	Audits transfer documentation, accompanying-staff fitness and stop-work events.

8. Quality monitoring (RCA → CAPA)

What is monitored:

- Sample of transfer-in records shows planned/unplanned status and clinical information on arrival.
- Sample of transfer-out/referral records shows stabilisation, mode, equipment, monitoring, and documented gaps where requirements were not met.
- Accompanying staff match clinical condition; doctor accompanies unstable admitted transfers in the sample.
- Transfer or discharge summary (including LAMA where applicable) given and copy retained.
- Stop-work events (held unsafe transfers) are logged with outcome.

Root-cause analysis is required when an unstable transfer proceeds without criteria, stabilisation or appropriate accompanying staff.

This policy is reviewed annually, and sooner after any adverse event during transfer.

9. Training and staff acknowledgement

Doctors, nurses and technologists who accompany transfers, emergency staff and medical records staff are informed of this policy at induction and once a year after that. Training covers planned/unplanned transfer-in, stabilisation and documentation of gaps, accompanying-staff rules including doctor for unstable patients, CPR expectations, transfer/discharge summaries including LAMA, and stop-work.

Staff acknowledgement

I have read the Policy on Transfer In, Transfer Out and Referral of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with clinical induction records.)

10. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: emergency; every ward and ICU; day-care; ambulance coordination; medical records; registration; nursing administration.

The current version is available to all staff at the clinical policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

11. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care (NABH Hospitals chapter)
CAPA	corrective and preventive action
CPR	cardiopulmonary resuscitation
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
ICU	intensive care unit
LAMA	left against medical advice
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element

12. Traceability to NABH HCO Full Accreditation 6th Edition AAC.3

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
AAC.3.a	Commitment*	Transfer-in of patients to the organisation is done appropriately.	Section 3; 5.1	Emergency In-Charge (unplanned); department heads (planned); treating doctors (receive); registration
AAC.3.b	Commitment*	Transfer-out / referral of patients to another facility is done	Section 3; 5.2; Section 6 Stop-work	Treating doctors (authorise); accompanying clinical staff (execute);

		appropriately.		Medical Superintendent (accountable)
AAC.3.c	Commitment	During transfer or referral, accompanying staff are appropriate to the clinical condition of the patient.	Section 3; 5.3; Section 6 Stop-work	Treating doctors (judge stability and assign); accompanying nurses/technologists (execute)
AAC.3.d	Commitment	The organisation gives a summary of the patient's condition and the treatment given.	Section 3; 5.4	Treating doctors (write/approve summary); Medical Records (retain copy)

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.3.a — Transfer-in done appropriately

- Transfer-in records showing planned or unplanned status and clinical information received.
- Ambulance crew or escort assignment notes for unplanned/suspected unstable transfer-ins where used.
- Feedback notes to referring organisation/doctor where provided.
- Registration of transfer-in patients under the unique identification number.

AAC.3.b — Transfer-out / referral done appropriately

- Transfer-out/referral orders with patient/family consultation noted where consulted.
- Documentation of pre-transfer stabilisation, mode/vehicle, equipment and monitoring.
- Documented reasons when stated transfer requirements could not be met.
- Log of transfers for emergency, admitted patients and diagnostic shifts.

AAC.3.c — Accompanying staff appropriate to clinical condition

- Transfer records naming accompanying staff and role.
- Evidence of CPR training (basic or advanced as appropriate) for accompanying staff.
- Records showing a doctor accompanied unstable admitted patients transferred out or shifted for diagnostics.
- Written stability/transfer criteria used to decide accompanying level.

AAC.3.d — Summary of condition and treatment given

- Transfer summaries for emergency and diagnostic/therapeutic transfers.
- Discharge summaries for patients discharged and transferred out, including LAMA.
- Retained copies filed under the unique identification number.

- Audit sample confirming summary given before departure.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.3.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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